

PONTALON® CAPSULE

A non-narcotic, well-tolerated analgesic with anti-inflammatory properties, Pontalon provides greater relief of pain than conventional analgesics. The potent anti-inflammatory property (five times greater than those of Acetyl-salicylic Acid) helps to broaden and extend the usefulness of Pontalon. Among the desired qualities of an analgesic agent are rapid onset and prolonged duration of action. With Pontalon, pain relief occurs within an hour after administration and is maintained for up to six hours.

Description:

A #2 pale green cap and white body hard gelatine capsule containing white powder, with a mark "PLC" printed on the capsule.

Ingredient(s):

Each capsule contains:

Mefenamic Acid 250mg

Pharmacodynamic:

Pontalon is a non-steroidal agent which demonstrated anti-inflammatory, analgesic and antipyretic activities. It inhibits the enzyme cyclo-oxygenase, resulting in decreased formation of precursors of prostaglandins and thromboxanes from Arachidonic Acid. It has also been shown to inhibit competitively the actions of prostaglandins. These actions may contribute significantly to its therapeutic effects.

Pharmacokinetic:

It is excreted in urine and feces both as the unchanged drug and as metabolites. Metabolized by liver. Half-life = 3 - 4 hours.

Indication(s):

For the relief of mild to moderate pain including muscular, traumatic and dental pain, headache, postoperative and postpartum pain, and dysmenorrhea.

Dosage and Administration:

Adults: The recommended initial dose is 500mg followed by 250mg every 6 hours as required and usually not to exceed one week. For dysmenorrhea, the recommended dosage is 500mg as an initial dose followed by 250mg every 6 hours, starting at the onset of menstrual pain and should not exceed for more than 2 – 3 days. Safe use of this drug in pregnancy has not been established.

After assessing the risk/ benefit ratio in each individual patient, the lowest effective dose for the shortest possible duration should be used.

Route of Administration: Oral

Contraindication(s):

1. Pontalon should not be used in patients who have previously exhibited hypersensitivity to it.
2. Pontalon is contraindicated in patients with active ulceration or chronic inflammation of either the upper or lower gastrointestinal tract.
3. Pontalon should be avoided in patients with pre-existing renal disease.

Precaution(s) / Warning(s):

Side effects are negligible at recommended dosage.

1. All NSAIDs can cause gastrointestinal discomfort and rarely serious, potentially fatal gastrointestinal effects such as ulcers, bleeding and perforation which may increase with dose or duration of use, but can occur at any time without warning. Caution is advised in patients with risk factors for gastrointestinal events e.g. the elderly, those with a history of serious gastrointestinal events, smoking and alcoholism. When gastrointestinal bleeding or ulceration occurs in patients receiving NSAIDs, the drug should be withdrawn immediately. Doctors should warn patients about signs and symptoms of serious gastrointestinal toxicity. The concurrent use of aspirin and NSAIDs also increases the risk of serious gastrointestinal adverse events.
2. NSAIDs may very rarely cause serious cutaneous adverse events such as exfoliative dermatitis, toxic epidermal necrolysis (TEN) and Stevens-Johnson Syndrome (SJS), which can be fatal and occur without warning. These serious adverse events are idiosyncratic and are independent of dose or duration of use. Patients should be advised of the signs and symptoms of serious skin reaction and to consult their doctor at the first appearance of a skin rash or any other sign of hypersensitivity.
3. If diarrhea occurs, the dosage should be reduced or temporarily suspended.
4. It should be used with caution in patients with impaired renal or liver function.
5. It has marked tendency to induce grand mal convulsions in overdosage and should be avoided in epileptic subjects.
6. Observational studies have indicated that non-selective NSAIDs may be associated with an increased risk of serious cardiovascular events, principally myocardial infarction, which may increase with dose or duration of use. Patients with cardiovascular disease or cardiovascular risk of an adverse cardiovascular event in patient taking NSAID, especially in those with cardiovascular risk factors, the lowest effective dose should be used for the shortest possible duration. There is no consistent evidence that the concurrent use of aspirin mitigates the possible increased risk of serious cardiovascular thrombotic events associated with NSAID use.
7. NSAIDs may lead to the onset of new hypertension or worsening the pre-existing hypertension and patients taking anti-hypertensive with NSAIDs may have an impaired anti-hypertensive response. Caution is advised when prescribing NSAIDs to patients with hypertension. Blood pressure should be monitored closely during initiation of NSAID treatment and at regular intervals thereafter.

8. Fluid retention and oedema have been observed in some patients taking NSAIDs, therefore caution is advised in patients with fluid retention or heart failure.
9. Skin Reactions: Serious skin reactions such as Generalised bullous fixed drug eruption (GBFDE) have been reported very rarely in association with the use of mefenamic acid. Mefenamic acid should be discontinued at the first appearance of the skin rash, mucosal lesions or any other sign of hypersensitivity.

WARNINGS

RISK OF GI ULCERATION, BLEEDING AND PERFORATION WITH NSAID

Serious GI toxicity such as bleeding, ulceration and perforation can occur at any time, with or without warning symptoms, in patients treated with NSAID therapy. Although minor upper GI problems (eg. dyspepsia) are common, usually developing early in therapy, prescribers should remain alert for ulceration and bleeding in patients treated with NSAIDs even in the absence of previous GI tract symptoms.

Studies to date have not identified any subset of patients not at risk of developing peptic ulceration and bleeding. Patients with prior history of serious adverse events and other risk factors associated with peptic ulcer disease (eg. alcoholism, smoking, and corticosteroid therapy) are at increased risk. Elderly or debilitated patients seem to tolerate ulceration or bleeding less than other individuals and account for most spontaneous reports for fatal GI events.

Drug Interaction:

Mefenamic Acid enhances the effect of Warfarin. Therefore, where given to a patient receiving Warfarin, the dosage of Warfarin should be reduced to prevent excessive prolongation of the prothrombin time.

Aspirin administered concurrently may lower Mefenamic Acid plasma level, possibly by competing for protein-binding sites. The urinary excretion is unaffected by Aspirin, indicating no change in absorption.

Mefenamic Acid does not affect serum Salicylate levels. Greater fecal blood loss results from concomitant administration of both drugs than from either drug alone.

Side Effect(s) / Adverse Reaction(s):

The commonest adverse effects occurring with Mefenamic Acid are gastrointestinal disturbances such as: diarrhea, nausea, vomiting and abdominal pain; other gastrointestinal adverse reactions occur less frequent are: anorexia, pyrosis, flatulence and constipation. Peptic ulceration and gastrointestinal bleeding have also been reported. Hypersensitivity reactions including skin rashes and urticaria, and occasionally allergic glomerulonephritis may occur. Asthma may be precipitated. Other side effects including headache, drowsiness, dizziness, nervousness, insomnia, facial edema and blurred vision, renal failure, including papillary necrosis have been reported. In elderly patients renal failure has occurred after 2-6 weeks of treatment. Renal failure may not be completely reversible. Hematuria and dysuria have also been reported. Skin and subcutaneous tissue disorders: Generalised bullous fixed drug eruption (GBFDE).

Symptoms and Treatment for Overdosage and Antidote(s):

Treatment of overdosage consists of emptying the stomach via induction of emesis or gastric lavage, administration of activated charcoal and antacids, and monitoring and supporting vital functions. Induction of diuresis may be useful in overdosage with it.

Shelf-Life:

3 years from the date of manufacture.

Storage Condition(s):

Keep in a tight container. Store at temperature below 30°C. Protect from light and moisture.

Packing:

Plastic bottle of 200's, 1000's and 1500's. (for export only)

Blister packing of 10's x 10, 10's x 120 and 10's x 100.

PONTALON® KAPSUL

Diskripsi:

Kapsul #2 dengan cap berwarna hijau muda dan badan berwarna putih, mengandung serbuk putih di dalamnya dengan tanda "PLC" dicetak pada kapsul.

Bahan Aktif:

Setiap kapsul mengandung:

Mefenamic Acid 250mg

Indikasi:

Untuk melegakan kesakitan otot, trauma, gigi, kepala, postoperasi, postpartum dan dismenorea.

Dos:

Dewasa: Dos permulaan 500mg diikuti 250mg setiap 6 jam bila perlu dan tidak melebihi satu minggu. Bagi dismenorea, dimakan bila kesakitan haid dan tidak melebihi 2 – 3 hari.



Manufacturer and Product Registration Holder:
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